过程性监控：

正确生成的脚本字数%	正确输出的剧本总字数/剧本的总字数*100%	被判定为正确：剧本中字的发音错误不超过一个明显的音发错误（替换、变形、添加或遗漏）	正常： 100%	Youmans G, Youmans S R, Hancock A B. Script training treatment for adults with apraxia of speech[J]. American Journal of Speech-Language Pathology, 2011, 20: 23-37. https://www.unm.edu/~atneel/shs531/youmans_script.pdf	were then analyzed, and the dependent variables of percentage of script words produced correctly (PSC), errors, and speaking rate (defined below) were measured for each of the scripts at the beginning of each of the sessions. A data point for each of the dependent variables was plotted on each graph for every session for visual inspection. PSC was measured by dividing the total number of words in the script that the participant produced correctly by the total number of words in the script and multiplying the quotient by 100. PSC was a measure of each participant's accuracy of script production and as such was the primary behavior of interest. Accuracy for PSC for purposes of initial scoring and reliability scoring was defined as script word productions having no more than one obvious sound production error (substitution, distortion, addition, or omission). In addition, if the one error changed the meaning or distorted the production so that it was judged unintelligible by the rater, the utterance was considered an error rather than a script correct utterance. Error production was also a variable of interest. Errors were defined as word or phrase repetitions that were deemed as noncommunicative, pauses greater than 3 s, unintelligible utterances, and interjections. Errors were viewed as a reflection of how much the participant struggled while attempting to produce scripts. The error count was not simply an inverse of words correct. A client could produce many errors (pauses, repetitions, etc.) or no errors, within any given script production attempt. Speaking rate in words per minute was calculated for each of the scripts during each session. Speaking rate was calculated by summing the number of nonerror, communicative words produced during the script attempt, whether part of the script or not. This count was then divided by the number of minutes of the production attempt.
错误	被认为没有交流意义的单词或短语重复、超过 3 秒的停顿、难以理解的发音以及插入语		正常： 0		
言语速率	总字数/说话时间	被确定为字：非错误、具有交流意义（排除“呃”“啊”等插入语，无意义的重复，反复自我修正）	正常： 180-240 字/min		

响度	正常： 60-78 dBA	University of Sydney Voice Lab. Acoustic Analysis Normative Data Reference Sheet: V4.0[Z]. Sydney: University of Sydney, 2025.	Intensity	The volume of voicing produced by an adult speaker.	Available Intensity Range (AIR): 59.81–97.53dB Conversational Intensity Range (CIR): 60.39–77.23dB Conversational Intensity Level (CIL): 68.15dB Maximum intensity: *Males:* Mean: 110dB SD: 7.1 *Females:* Mean: 106dB SD: 3	Gelfer, M.P., & Young, S.R. (1997). Comparisons of intensity measures and their stability in male and female speakers. *Journal of Voice*, 11(2), 178–186. doi:10.1016/s0892-1997(97)80076-8. Casper, J.K., & Leonard, R. (2006). *Understanding voice problems: A physiological perspective for diagnosis and treatment*. Lippincott Williams & Wilkins.

训练前后对比：

WAB量表	自发言语 信息量 流利性，语法能力和语序 可懂度	现将西方失语症成套测验(WAB)全文介绍如下： **1 自发言语** 用具(风景)卡 No. 1 将病人的言语记录在纸上并录音。如必要可用熟悉的问题代替。答案的流利性和信息量根据下面的标准给分。 ①你今天好些吗？ ②你以前到过这儿吗？ ③你叫什么名字？ ④你住在哪儿？ ⑤你做什么工作？ ⑥说说你为什么到这儿来，好吗？ 或你有什么不舒服吗？ ⑦看图说话 将一幅郊游风景画(如图1)摆在病人面前并说："说说你看到了什么，试着用句子说。"鼓励病人注意画的各个方面。将画移至病人正常视野内。如病人只说出了几个词，要设法让病人作出他力所能及的最好回答。  图1 郊游风景画	王荫华.西方失语症成套测验(WAB)介绍(一)[J].中国康复理论与实践,1997,(2)87-89.

	复述 可懂度	分数 ①床 2 ②鼻子 2 ③烟斗 2 ④窗户 2 ⑤香蕉 2 ⑥雪球 4 ⑦45 4 ⑧95% 6 ⑨62.5 10 ⑩电话响着呢 8 ⑪他不回来了 10 ⑫作糕点令人兴高采烈 10 ⑬英国第一门野战炮 8 ⑭没有，假如，和，或，但是 10 ⑮把5打饮料罐的装璜 装进我的盒子 20 最高分 100	王荫华.西方失语症成套测验(WAB)介绍(二)[J].中国康复理论与实践,1997,(3):135-140.

<table>
<tr>
<td>CADL 量表</td>
<td>
附录 8：疗效评价-日常生活交流能力量表（CADL）

日常生活交流能力（CADL）评定和记分
<table>
<tr><th>顺序</th><th>项目</th><th>项目内容</th><th>得分</th><th>0</th><th>1</th><th>2</th><th>3、4</th></tr>
<tr><td>1</td><td>1</td><td>适当的问候</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>2</td><td>2-①</td><td>表明自己的情况（姓名）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>3</td><td>2-②</td><td>表明自己的情况（是-不是）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>4</td><td>2-③</td><td>表明自己的情况（住址）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>5</td><td>2-④</td><td>表明自己的情况（年龄）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>6</td><td>3</td><td>要求对方重复问题</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>7</td><td>4</td><td>说出自己的症状</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>8</td><td>5-①</td><td>填写出诊单（姓名、住址、年龄）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>9</td><td>5-②</td><td>填写出诊单（症状）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>10</td><td>5-③</td><td>填写出诊单（抄写病案号）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>11</td><td>6-①</td><td>看懂院内标志（内科、外科）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>12</td><td>6-②</td><td>看懂院内标志（中、西药房）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>13</td><td>7</td><td>按规定量服药</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>14</td><td>8</td><td>小卖部买冷饮</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>15</td><td>9</td><td>辨明楼层</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>16</td><td>10-①</td><td>购物（选择物品）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>17</td><td>10-②</td><td>购物（判断价格）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>18</td><td>10-③</td><td>购物（计算找钱）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>19</td><td>11</td><td>看菜单点菜</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>20</td><td>12-①</td><td>问路（向交警问路）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>21</td><td>12-②</td><td>问路（理解走向顺序）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>22</td><td>13</td><td>理解指令</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>23</td><td>14-①</td><td>打电话（拨电话）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>24</td><td>14-②</td><td>打电话（说要求）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>25</td><td>15</td><td>查电话号码</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>26</td><td>16-①</td><td>接电话</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>27</td><td>16-②</td><td>接电话（记下电话内容）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>28</td><td>17</td><td>对表</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>29</td><td>18</td><td>报告时间</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>30</td><td>19-①</td><td>看电视节目表（选择节目）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>31</td><td>19-②</td><td>看电视节目表（选择频道）</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>32</td><td>20</td><td>读报</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>33</td><td>21</td><td>听电台的天气预报</td><td></td><td></td><td></td><td></td><td></td></tr>
<tr><td>34</td><td>22</td><td>理解量的概念</td><td></td><td></td><td></td><td></td><td></td></tr>
</table>

CADL 评分标准
4 分　在与家属以外的人员交谈时，一般均可做出适当反应；
3 分　超过 3~30 秒才作出反应，其实用性未达到 4 分的水平；
2 分　给予提示或反复刺激方达到 4 分水平；
1 分　给予提示或反复刺激方达到 3 分水平；
0 分　回答完全错误。
</td>
<td>王哲.基于韵律功能的脚本训练对非流畅性失语症言语可懂度的干预[D].华东师范大学,2024.DOI:10.27149/d.cnki.ghdsu.2024.000514.</td>
</tr>
<tr>
<td>喜好度与接受度</td>
<td></td>
<td></td>
</tr>
</table>